



Fiona Stanley Fremantle Hospitals Group

Guideline

Eating disorders assessment, admission, and initial management: Emergency Department, Acute Medical Unit, General Medical Ward 6C or Psychiatric Wards

Reference #: FSFH-HW-GUI-0001

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital	Emergency Department, Acute Medical Unit, General Medical Ward 6c or Psychiatric wards	Medical, Nursing, Midwifery & Allied Health

1. Introduction

Individuals presenting with symptoms suggestive of an eating disorder to Fiona Stanley Hospital require comprehensive physical and mental health assessment. Where an eating disorder is suspected early intervention and optimisation of nutrition is fundamental to improving patient outcomes.

This document provides a guide to the clinical service options available for this patient group and their family with the aim of improving identification and management of the psychological risks, improving prognosis, and reducing the impact of the disease through the coordinated care of Medical and Mental Health services.

2. Terminology

Eating Disorders

A complex group of illnesses that can be life threatening and can present with a range of serious physical health issues. Eating Disorders are serious mental health disorders, often with other psychiatric comorbidities. Treatment needs to be provided with a collaborative approach between both medical and mental health services.

The information provided in this document is based on its relevance to Fiona Stanley Fremantle Hospital Group (FSFHG) only. Due to differences in context, scope of practice and differences in service delivery, FSFHG does not make claim to its relevance or appropriateness for use within other organisations/sites.

West Australian Eating Disorder Outreach Consultation Service (WAEDOCS)	State-wide consultation service that aims to ensure youths and adults with an eating disorder can access best practice care.
Body Mass Index (BMI)	A person's weight in kilograms divided by the square of the person's height in meters (kg/m ²)
Postural Orthostatic Tachycardia Syndrome (POTS)	Type of dysautonomia, characterized in adults by increased heart rate of at least 30 beats per minute within ten minutes of standing up, occurred in the absence of orthostatic hypotension with or without symptoms; in children and teenagers, the difference is above 40-50 beats per minute; high variability among individuals
Refeeding syndrome	Metabolic disturbance that occurs because of reinstitution of nutrition in people who are starved or severely malnourished, during the initial four to seven days following re-feeding. Re-starting intracellular production of glycogen, fat and proteins may cause low serum concentrations of potassium, magnesium, phosphate, glucose, amino acids, etc. with further possible cardiac, pulmonary and neurological symptoms.
Orthostatic hypotension	Drop in blood pressure – for the systolic at least 20mmHg and diastolic at least 10mmHg – within 3 minutes of standing.
MHEDLS	Mental Health Emergency Department Liaison Service

3. Care coordination and patient governance

- Coordinated care for patients with an Eating Disorder aims to optimise treatment by:
 - improving /shortening illness
 - reducing morbidity and mortality
 - promoting integrated care
 - seamless service provision across medical and mental health services
 - involving family/carers whenever possible.
- Overall governance of the patient remains with the admitting Medical/Psychiatric Consultant depending on the setting where care is delivered. The patient care team includes:
 - Mental Health Consultation-Liaison Service (MHCLS) which provides multi-disciplinary mental health support to the patient and treating team when they are admitted to a Medical ward or Intensive Care Unit. MHCLS includes Consultation-Liaison psychiatry, clinical psychology, mental health nursing and neuropsychology
 - Nurse Unit Manager from ward where patient is currently admitted (or proxy)
 - Dietitian
 - Other Allied Health staff as required.

At FSFHG eating disorders are managed in accordance with the WAEDOCS guideline [Eating Disorders: the Management of Youth and Adults – a Quick Reference Guide](#)

4. Emergency Department initial assessment

4.1. Assessment including patient history

A comprehensive patient assessment will be conducted on presentation or identification of a patient with disordered eating. Include family members/carers where possible in the assessment process. And liaison with current care providers.

BMI should never be the sole determinant when assessing risk in a person with an eating disorder. The patient will require a thorough medical and psychiatric assessment

4.1.1. Nursing Assessment

- Complete the following observations:
 - Supine and erect blood pressure and heart rate.
 - ECG
 - Blood Glucose Level (BGL)
 - Weight (post void, pre diet & fluids, in hospital gown and underwear only) and height
 - BMI
- Commence the following documentation:
 - WA Eating Disorders Outreach & Consultation Service BMI / Weight Chart
 - Fluid Balance Chart
 - Dietary Intake Record
 - Bowel Chart
 - Eating Disorder Patient Care Plan
- Review patient risk factors and behaviour. If applicable commence Behavioural Observation Chart:
 - refer to the FSFHG Specialising and Observations: Provision of a special for patients in non-Psychiatric areas Guideline
- If within business hours arrange Dietetics review. If after hours, ensure correct diet/allergy requirements are entered into Allergy and Diet Application (ADA) and that a Dietetics e-Referral is made.

4.2. Medical assessment

- General clinical examination and
 - Postural observations – heart rate and blood pressure
 - ECG
 - Collect bloods for Pathology: full blood count with differentials, urea & electrolytes, calcium, magnesium, phosphates, liver function tests, albumin, haematinics, vitamin D, thyroid function tests
- Signs of medical instability:
 - symptomatic POTS, haemodynamic significant brady/tachyarrhythmia, prolonged QTc interval on ECG
 - symptomatic orthostatic hypotension

- Note that there is a high variability among individuals with regards to POTS, orthostatic hypotension, and their symptoms (individuals with long-standing high POTS may be asymptomatic, while others with minor or moderate values might be highly symptomatic)
- hypoglycaemia
- significant electrolyte disturbance

Telemetry is indicated in the following circumstances:

- Haemodynamically significant tachycardia or bradycardia
- Symptomatic postural orthostatic tachycardia syndrome
- Symptomatic/significant (≤ 80 mmHg) (orthostatic) hypotension
- Significant hypokalaemia, hypocalcaemia, hypomagnesaemia
- QTc interval > 500 msec on ECG (if QTc 460-500msec – daily ECG)
- History of pre-existent arrhythmia or other heart disease
- Family history of congenital long QT syndrome or familial channelopathies
- Loss of consciousness from unclear cause

4.3.Mental Health assessment

- Refer patient to the Mental Health Emergency Department Liaison Service (MHEDLS)
- MHEDLS will:
 - complete and document a Mental Health Triage & Risk Assessment and Management Plan (RAMP)
 - Consider the least restrictive options for treatment and appropriateness of voluntary or involuntary admission under the Mental Health Act 2014
- Assess need for a special.
 - Refer to the FSFHG Specialising and Observations – Provision of a Special for Patients in Non-Psychiatric Clinical Areas guideline
- Provide advice on management and treatment related to the psychological factors impacting on the person.
- Refer to the FSFHG Medication Management of Agitation & Arousal in Mental Health Inpatients policy
- When the patient is transferred to AMU/6C prior to MHEDL review the patient will be seen by the MH Consultation Liaison Service once eReferral received.

5. Management on Acute Medical Unit, General Medical Ward 6C or ICU

- Suitable patients for medical admission are those with medical instability.
 - Medically stable patients are admitted to Mental Health ward if admission is required (as per MH Team assessment in ED), or discharged into the community with local network follow-up
- Patients to be admitted to either AMU or General Medicine Ward 6C, as required by the need of telemetry or bed situation
- Acute medically unstable patients are to commence on the Nutritional management pathway for acute medically unstable eating disorder patients (as per [Appendix 1](#))
- Patients with severe, life threatening dyselectrolytaemia, arrhythmia, hypotension or seizures, should be admitted to ICU/HDU.
- Refer to: [Appendix 3](#) eating disorder admission flow chart
- Goals of medical care include:
 - haemodynamic stabilisation
 - prevention and treatment of re-feeding syndrome
 - safe nutritional and weight restoration
 - reversal of acute cognitive effects of starvation
 - Planning for discharge or transfer to next step of staged care once Medical treatment completed

5.1. Dietetics management.

- Nutrition management to be commenced by nursing and medical as per the Nutritional management pathway for acute medically unstable eating disorder patients please refer to [Appendix 1](#)
- Complete a full nutritional assessment within 72 hours of admission unless otherwise indicated
 - Earlier assessment may be required if BMI <13 or complex comorbidities present that may deem patient unsuitable for eating disorder pathway (ie. renal impairment, confirmed milk allergy, gut failure)
- Calculate nutrition requirements and guide BMI targets
- Provide a suitable Nutritional Management Plan for safe nutrition restoration

- Facilitate transition to oral meal plans that meet nutrition targets to encourage oral intake prior to discharge
- Provide regular psychoeducation as appropriate
- Facilitate commencement of oral meal plans that meet nutrition targets to encourage oral intake prior to discharge to assist with transition to the community

5.2.Nursing management.

- On arrival to ward insert NGT to facilitate safe nutritional restoration in the acute phase.
- If within business hours arrange Dietetics review.
 - If after hours ensure correct diet/allergy requirements are entered in Allergy and Diet Application (ADA) and a dietetics e-Referral is made, and commence the Eating Disorders After Hours Nasogastric Tube Feeding Regime ([Appendix 1](#))
- Maintain the following documentation:
 - Adult Observation & Response Chart, Maternity Adult Deterioration Detection System Chart (MADDS)
 - WA Eating Disorders Outreach & Consultation Service BMI / Weight Chart
 - Fluid Balance Chart
 - Dietary Intake Record
 - Bowel Chart
 - Eating Disorder Patient Care Plan
 - Insulin Subcutaneous Order and Blood Glucose Record - Adult
- Complete the following observations:
 - general observations QID or more frequently as clinically indicated
 - QID supine and erect blood pressure and heart rate
 - daily ECG
 - BSL QID two hours post prandial (post meals) and at 0200 if requested by Medical team
 - daily blind weight (post void, pre diet & fluids, in hospital gown and underwear only, same scales)
 - daily BMI
 - strict food and fluid intake and output monitoring; urine specific gravity if water overload is suspected
 - four hourly gastric residual volume checks for patients on continuous NGT feeds

- auscultation for bowel sounds each shift
- Review patient risk factors and behaviour and if applicable commence Behavioural Observation chart.
 - Refer to the FSFHG Specialising and Observations – Provision of a Special for Patients in Non-Psychiatric Clinical Areas guideline for further details
- Ensure nutritional replenishment is maintained in accordance with the Dietetics plan including fluid restriction and caffeine restriction
- Ensure mobility and exercise allowances/restrictions are maintained in accordance with the Medical plan
- Ensure leave allowances and risk management strategies are maintained as per to the MHCLS plan
- If urgent Mental Health input is needed contact MHCLS within business hours, and the Mental Health ED Registrar after hours
- For patients identified as a high risk of medication diversion or at risk of stockpiling medications, the following strategies may be considered:
 - All doses of medication administration should be directly observed by the registered nurse. The nurse must watch the patient swallow the dose, encourage them to drink some water post dose and engage in a short conversation after swallowing to ensure the dose is not retained in the mouth.
 - Alternative formulations such as wafers, orally dispersible tablets or liquids may be considered if clinically appropriate. Please consult consultation-liaison psychiatry or pharmacy for advice if required.
 - Patient property/drawers may have to be searched under the guidance of Security if there is a high suspicion of stockpiling, diversion of medications
- For patients identified at risk of absconding, self-harm, or harming others, the patient's property/drawers should be searched by two staff members (and security if needed) on arrival to ward and at least once per day.
- Self-harm tools may include sharp objects for cutting (i.e., razor blades, broken pens, phone screen glass, sharps container) and/or cords which could be used for ligature (i.e., pulse-oximeter, naso-gastric and feeding tubes, suction tubing, electronic charging cables, call bell, PES remote, patient clothing)
- Patient belongings assessed to pose risk to the individual patient will be stored immediately outside of the patient room under the supervision of nursing staff
- Verbal de-escalation followed by code black/restraining plans should be made in advance.

5.3. Medical management

- Continue monitoring for and correcting medical instability, including the presence of symptomatic postural orthostatic tachycardia, brady/tachyarrhythmia, prolonged QTc interval on ECG, orthostatic hypotension, hypoglycaemia, & electrolyte imbalances.
- Note
 - significant individual variability – the trend is more important than the actual value of the POTS, etc! Teenagers have a higher ‘normal’ variability of BP and HR in orthostatism.
 - Most of the haemodynamic parameters get normalised in months of adequate re-feeding, so their eradication should not be a goal in itself, it is most often not achieved during admission.
 - ‘POTS treatment’ is actually re-feeding – normal nutrition improves the autonomic dysfunction with time.
 - POTS or hypotension need separate treatment (e.g., fludrocortisone, IV hydration, betablocker, ivabradine, etc) only if the patient is severely symptomatic – e.g., at high risk of collapse
- Daily assessment of blind weights/BMI, and their trend. Disclosure of weight or weight trends can be very distressing for the patient and may interfere with the treatment efficiency.
- Daily assessment of ECG in the first week, then as indicated by the Team (usually weekly)
- In the Acute Medical Unit telemetry should be provided as indicated in Section 4.2
- Prescribe daily supplementation as per the [WAEDOCS guideline](#)
- Liaise with MHCL Service and ward NUM regarding specialising and observation requirements e.g., supervision in the bathroom, and/or during off ward leave
- Determine physical mobility and exercise allowances/restrictions according to medical stability
- Monitor nutritional replenishment in consultation with Dietetics staff
- Monitor for Refeeding Syndrome:
 - Daily bloods: Potassium, Calcium, Magnesium, Phosphates – first week
 - 2nd daily liver function tests – if progressively altered request liver ultrasound for early diagnosis of fatty liver of refeeding
 - From the second week – twice or once weekly as appropriate

- Treat symptoms as nausea or constipation only if they are severe and put the patient at risk of aspiration; they are most likely caused by the autonomic dysfunction of eating disorder and will subside with nutrition restoration. Two stools in the first week are normal in this setting. The excess of aperients might worsen a previous laxative abuse.
- Liaise with MHLC, Nursing and Dietetics in weekly multidisciplinary team meetings.

5.4.MHCLS management

- Review the patient for the purpose of assessment, diagnosis and psychiatric treatment
- Provide psychoeducation regarding eating disorder risks and recovery
- Support patient and family, Medical, Nursing and MDT staff
- Provide management and advice regarding distress/anxiety during refeeding process
- Conduct regular reviews based on clinical need (Monday to Friday) to assist in developing multidisciplinary management plan
 - Monitor progress and liaise with the Medical team.
- Determine any restrictions necessary from a psychiatric perspective to ensure safe nutritional restoration, for example suitability for leave from ward, level of observation required, bathroom access.
 - Liaise with ward NUM and treating Medical team regarding need for a companion
- Assess suitability for voluntary or involuntary care as per requirements of the Mental Health Act 2014.
- Undertake discharge/transfer planning, including liaison with relevant psychiatric inpatient / outpatient services.
- Also refer to [Appendix 2](#) "Role of the FSH MHCL Clinical Psychologist in Eating Disorder Management"

5.4.1. Complex patients

For patients identified by the treating team as highly complex with regards to their management or transfer/discharge to other services, it is recommended that a combined Executive-led case conference be conducted as soon as practicable.

5.4.2. Discharge planning

- Refer to [Appendix 3](#).
- Refer to [Appendix 4](#) (Eating Disorders Decision Tree and Service Options)
- MHCLS aims to work with the patient and their family towards suitable discharge planning including referrals to relevant mental health services as required
- MHCLS will provide clinical handover to the accepting Mental Health team when the patient is waitlisted for a bed at the FSH Mental Health Unit or Fremantle Hospital psychiatric wards at the time of transfer.

6 Compliance/performance monitoring

- The FSFHG Mental Health Service Nurse Director will be responsible for monitoring compliance with this document via routine clinical incident review processes

7 Related policy documents

FSFHG:

- Specialling and observations: Provision of a special for patients in non-Psychiatric clinical areas (FSFH-HW-GUI-0045)
- Medication Management of Agitation and Arousal in Mental Health Inpatients (FSFH-MENH-GUI-0004)

NMHS:

- WAEDOCS Clinical guideline: [Eating Disorders: the Management of Youth and Adults – a Quick Reference Guide](#)

8 Other related documents

The Mental Health Act 2014

9 Related standards

NSQHS Standards:

- Clinical Governance
- Comprehensive Care
- Communicating for Safety

NSMHS Standards:

- Consumer and carer participation
- Delivery of care

10 Bibliography

Government of Western Australia, North Metropolitan Health Service: West Australian Eating Disorders Outreach and Consultation Service (WAEDOCS) (2016). *“Eating Disorders are Everybody’s Business”: Admission and Inpatient Treatment for Youth and Adults with Eating Disorders in Western Australia* (draft guidelines). Available by contacting WAEDOCS at waedocs@health.wa.gov.au or by phoning 1300 620 208.

New South Wales Ministry of Health: Centre for Eating and Dieting Disorders (CEED) (2014). *Guidelines for the Inpatient Management of Adult Eating Disorders in General Medical and Psychiatric Settings in NSW*. Available at: <https://www.health.nsw.gov.au/mentalhealth/resources/Publications/inpatient-adult-eating-disorders.pdf>

11 Authorisation

EXECUTIVE SPONSOR: Nurse Director Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	06/2017	Nurse Director, Mental Health	Eating Disorders Working Group	FSFHG Policy Committee	06/2020
2	06/2018	Nurse Director, Mental Health	Eating Disorders Working Group	FSFHG Policy Committee	06/2021
3	05/2020	Nurse Director, Mental Health	Eating Disorders Working Group	FSFHG Policy Committee	05/2024
4	10/2020	Nurse Unit Manager, General Medicine, Service 4, Acute Care Services	NMPGC	FSFHG Policy Committee	05/2024
5	09/2023	Nurse Director, Mental Health	NMPGC	FSFHG Policy Committee	10/2027

12. Appendices

12.1 Appendix 1. Nutritional management pathway for acute unstable eating disorder patients.

Nutritional management pathway for acute medically unstable eating disorder patients

Step 1	- Medical to determine suitability for eating disorder nutrition pathway (as per medical instability guidelines) - Referral to MHCLS - Call triage 21586 during business hours, OR - eReferral to Psychiatry -> Liaison Service – Adult - Recommend NGT insertion for all patients (do not wait for dietitian review) - use fine bore 12Fr, need placement confirmed prior to feeding - Medical to ensure prophylactic supplementation prescribed prior to commencing continuous feeds 300mg thiamine daily 1 multivitamin and mineral supplement daily 1 B complex supplement daily 500mg Phosphate Sandoz BD - Ensure deranged electrolytes are assertively replaced if required
Step 2	- Assess allergy status (not intolerances) or special dietary considerations (eg renal impairment or gut failure) and adjust plan in consultation with the Dietitian - place patient NBM on ADA - Referral to Dietitian - Call AMU/General Medical Dietitian on 72965 within business hours, OR - eReferral to General Medicine -> Dietitian after hours - oral water intake limited to: 250mL per day
Step 3	- Nasogastric feeds should commence ONCE tube placement confirmed, supplementation charted, and electrolyte replacement charted if required - Nasogastric feeds to commence at DAY 1 40mL/hr Fresubin Energy 24/24 + 50mL water flushes every 4/24 ** If Fresubin Energy is unavailable, please use isosource 1.5 - Refeeding bloods to be ordered by medical team daily for first 5 days
Step 4	Medical team or dietitian to determine if there are any contraindications to feed rate increasing on day 3 and day 5, if not DAY 3 60mL/hr Fresubin Energy 24/24 + 50mL h2o flush every 4/24 DAY 5 80mL/hr Fresubin Energy 24/24 + 50mL h2o flush every 4/24
Other Considerations	1x warm drink daily – limited to tea, coffee, milo. Provided by nursing only, made to standard dosing (ie 1tsp coffee per cup). To be included in 250mL oral fluid/day. NO food from home NO changes to nutrition management pathway without full MDT discussion

	Day 1	Day 3	Day 5
Date: Time started:			
Delivery	Pump (Kangaroo/Joey)	Pump (Kangaroo/Joey)	Pump (Kangaroo/Joey)
Product	Fresubin Energy	Fresubin Energy	Fresubin Energy
Feeding Period	24/24	24/24	24/24
Feed Rate (mL/hr)	40mL/hr	60mL/hr	80mL/hr
Water flush (mL)	50mL every 4/24	50mL every 4/24	50mL every 4/24
Total feed (mL)	960mL	1440mL	1920mL
Total flushes (mL)	300mL	300mL	300mL
Total fluid (mL)	1260mL	1740mL	2220mL
Comments			

Appendix 2. Role of FSH MHCL Clinical Psychologist in Eating Disorder Management

Introduction

This document summarises the role of clinical psychologists within the FSH Mental Health Consultation-Liaison Service (MHCLS), in working with patients admitted with an eating disorder.

The role is consistent with the recommendations of the West Australian Eating Disorders Outreach and Consultation Service (WAEDOCS) guidelines (2016)¹ and the NSW Health Centre for Eating and Dieting Disorders (CEED) guidelines (2014)².

Clinical Psychologists, in conjunction with the MHCLS and General Medical teams will offer:

- Comprehensive psychological assessment of patient, develop clear formulation, and offer evidence informed psychological interventions (individual/family based) and liaise with external services.
- Identify and manage the psychological risks and needs of patients with severe eating disorders.
- Optimise treatment including improving prognosis, shortening illness duration and reducing morbidity and mortality.
- Promote coordinated and integrated care with a smooth transition across medical, mental health, specialist, and community services with family involvement whenever possible.

A clinical psychologist, if available, should visit the patient ideally within 48 hours (WAEDOCS, 2016, draft guidelines pp, 30¹).

Role of Clinical Psychologists during Weight Restoration

Treatment of malnutrition and related medical instability is a clinical priority for patients presenting with starvation syndrome and/or who require refeeding. Concurrent clinical psychological interventions play a valuable at several levels including:

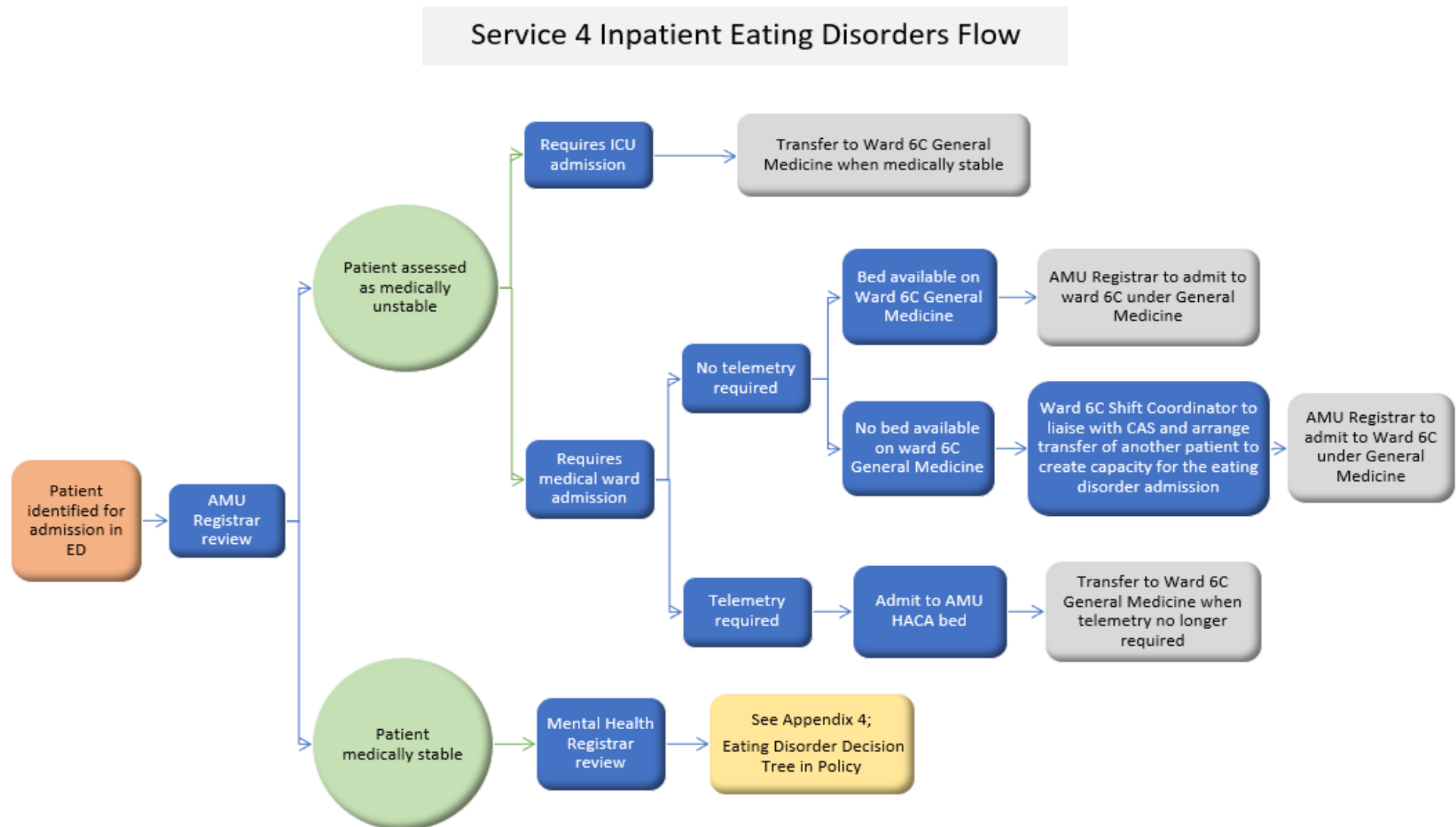
1. **Direct Patient interventions:** supportive psychological interventions aimed at improving engagement in treatment, managing fear of weight regain, distress tolerance, emotional containment, and provision of psychoeducation.
2. **Systemic interventions** with families/carers: emotional support and psychoeducation in relation to treatment, liaison between family and medical/psychiatric team to support understanding of treatment and of patient, gathering collateral information to assist with formulation and treatment planning.
3. **Consultation and Liaison with Medical MDT:** debriefing for staff working with challenging and high-risk behaviours, consultation on psychological issues, behavioural management, patient engagement and transference/countertransference issues.
4. **Liaison within the MHCL team:** consultation in relation to psychological/behavioural presentation, contribution to formulation and treatment planning, attendance at weekly MDT.
- 5.

Overarching aim is to ensure smooth flow of treatment between inpatient and outpatient services, and to develop a plan that ensures continuation of treatment should patient be re-admitted (recognising that recovery from AN is often a fluid process requiring step-up and step-down transitions between health care services).

References

1. Government of Western Australia, North Metropolitan Health Service: West Australian Eating Disorders Outreach and Consultation Service (WAEDOCS) (2016). *"Eating Disorders are Everybody's Business": Admission and Inpatient Treatment for Youth and Adults with Eating Disorders in Western Australia* (draft guidelines). Available by contacting WAEDOCS at waedocs@health.wa.gov.au or by phoning 1300 620 208.
2. New South Wales Ministry of Health: Centre for Eating and Dieting Disorders (CEED) (2014). *Guidelines for the Inpatient Management of Adult Eating Disorders in General Medical and Psychiatric Settings in NSW*. Available at: <https://www.health.nsw.gov.au/mentalhealth/resources/Publications/inpatient-adult-eating-disorders.pdf>

12.3 Appendix 3. Service 4 Inpatient Eating Disorder Flow



12.4 Appendix 4. Eating Disorders Decision Tree and Service Options

